

STATEMENT OF NEED (SON)

Independent Duty Medical Technician (IDMT) Advanced Procedural Skills Training

1.0 BACKGROUND

This requirement is for a one-day, high-fidelity procedural skills class designed to fill a critical training gap created by the removal of Tactical Combat Casualty Care (TCCC) from the Independent Duty Medical Technician (IDMT) curriculum. The course is a mission-critical necessity to bridge this validated training gap. The primary goal is to ensure every IDMT graduate possesses the foundational, hands-on competence and confidence to perform life-saving interventions in an operational environment. The estimated annual workload is **nine (9) course iterations per year**, starting in Fiscal Year 2027.

2.0 OBJECTIVE

The objective is to provide IDMT students with intensive, hands-on practice of core life-saving medical procedures using high-fidelity simulation. The course must build procedural muscle memory and clinical confidence immediately prior to students entering their real-world clinical rotations.

3.0 SCOPE

The contractor shall provide all non-personal services, including instructional faculty, facilities, equipment, simulators, and all supplies required to deliver the training outlined herein.

4.0 SPECIFIC REQUIREMENTS

4.1 Core Curriculum: The course must be structured around hands-on, rotational skills stations. The contractor shall provide hands-on training and evaluation for, but not limited to, the following core skills:

- **Hemorrhage Control:** Wound packing (including junctional sites), tourniquet application, and digital pressure techniques.
- **Airway Management:** Supraglottic airway insertion and confirmation, and performance of surgical cricothyrotomy.
- **Vascular Access:** Peripheral IV placement, saphenous vein cutdown, and intraosseous (IO) access (e.g., sternal, humeral, tibial).
- **Thoracic Injury Management:** Needle decompression of tension pneumothorax and chest tube insertion (thoracostomy).
- **Basic Surgical Skills:** Foundational suturing and knot-tying techniques for wound closure.

4.2 Location of Performance: The training facility must be located within a 30-mile radius of the Medical Education and Training Campus (METC) on Joint Base San Antonio-Fort Sam Houston to accommodate students lodging on base. A contractor-

provided mobile training solution may be considered but must operate within the fixed-space allocations of the government facility.

4.3 Instructor Qualifications & Ratios:

- **Qualifications:** Instruction must be led by qualified personnel with demonstrable expertise in emergency, trauma, or critical care medicine (e.g., Nationally Registered Paramedics, Physician Assistants, Physicians, or experienced U.S. military special operations medics). All instructors must be capable of providing hands-on supervision with immediate corrective feedback.
- **Ratios:** A 1:16 (instructor-to-student) ratio is acceptable for group instruction, but a **1:1 ratio is mandatory** during individual skills evaluation.

4.4 Training Fidelity:

- **Simulation:** All training must be conducted on high-fidelity simulation models capable of providing realistic tactile feedback, including task trainers for vascular access, surgical airway models with realistic tissue layers, and torso models that allow for proper decompression and chest tube insertion.
- **Training Plan:** Prior to the first session, the contractor shall deliver their detailed curriculum and evaluation checklists for Government approval.

4.5 Measurement of Student Proficiency:

- **Standard:** Student proficiency shall be measured against a formal, government-approved, standardized checklist for each core procedure. Evaluation will be based on a "**first-pass success**" standard, where the student must perform all critical steps correctly on the first attempt without coaching.
- **Failure Criteria:** The checklist must define specific **critical failure points** (e.g., incorrect landmark identification, violation of sterile technique). Committing a single critical failure will result in an immediate failure of the evaluation for that procedure.

5.0 LOGISTICS & PRICING

5.1 Class Size: The contractor must be capable of accommodating a class size of up to 16 students. The Government anticipates class sizes will realistically range from **8 to 14 students** per session.

5.2 Pricing Structure: Pricing shall be on a **Firm-Fixed-Price (FFP) per-student basis**. The contractor shall invoice the Government based on the actual number of students who attend each training session. This price shall be all-inclusive.

6.0 OPTIONAL DELIVERABLES

6.1 Individualized Feedback Report (Nice to Have): The Government has a strong interest in a system where instructors provide a written one-page report for each student, detailing procedural strengths and areas for improvement. Offerors should describe their ability to provide this.

6.2 Video Laryngoscopy: The Government has a strong interest in this course including an introduction and hands-on familiarization time with video laryngoscopy equipment.

6.3 Procedural Ultrasound: The Government has a strong interest in this course integrating procedural ultrasound applications, specifically for identifying vascular structures to aid in IV placement and confirming tube/airway placement.

7.0 CONTRACTOR PERSONNEL & QUALITY

7.1 Key Personnel: The Lead Instructor(s) shall be designated as Key Personnel and cannot be replaced without Government review and approval of the substitute's qualifications.

7.2 Quality Control Plan (QCP): The contractor shall establish and maintain a QCP detailing their methods for ensuring all requirements of this SoN are consistently met.

8.0 GOVERNMENT FURNISHED ITEMS

No Government Furnished Equipment, information, or resources will be provided.